



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : امال عبدالعظيم محمد عدوى

المحافظة : القاهرة الحي / المدينة : الوايلي

العنوان : 12ش غرب القشلاق-ح
البساطى-الوايلي-القاهرة

تاريخ تسجيل الإستمارة : 28-09-2026

رقم البطاقة : 27111131300741 رقم التليفون : 01112396683

العمر(تاريخ الميلاد) : 13-11-1971 المهنة : حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات الاروماتيز [14161]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: أمال عبدالمعظيم محمد عدوي National ID: 27111131300741	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

اسم
عبد العظيم محمد عدوى

١٢ ش غرب القشلاق - ح البساطي
الوايلي - القاهرة

٢٧١١١١٣١٣٠٠٧٤١

JQ7924108





Name: امال عبدالعظيم محمد عدوي		
MR. : 8266		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient's initial pathology report from December 2019 indicates a breast carcinoma that is estrogen receptor negative, progesterone receptor negative, and HER2/neu positive with a score of 3+. In May 2026, a CT scan of the chest revealed pulmonary nodules measuring 7 mm, prompting a follow-up plan. A subsequent breast ultrasound in April 2026 demonstrated free fluid with BIRADS 1 and BIRADS 2 findings. The patient was started on Femara (letrozole) with supportive therapy for a duration of three months, with the next clinic visit scheduled for 14 September 2026.		
تحريراً في ,		
	التاريخ :	20-09-2026
	الوقت :	نهى سعيد علي محمد
الطبيب المعالج :		
يعتمد ,		



Pathology Report

Name: أمال عبد العظيم محمد

Age: 48 years.

Clinical data: Breast mass.

Specimen type: Wide local excision and axillary lymph nodes.

Date: 20/7/2019

Lab. Number: 585

Hospital Number: 1973/2019

Gross Description:

A-sector of breast tissue; 12x7x4.5 cm, with attached axilla; 8x6 cm, received enblock. Serial cuts revealed an ill-defined firm indurated greyish white area; 3x2.5 cm involving hard mass 1.1x1 cm, located (3, 1.5, 1, 7, 1.3 & 1.2) cm from (upper, lower, medial, lateral, superficial & deep) margins respectively. Axillary dissection revealed 24 nodules up to 1.7 cm in diameter.

Microscopic Examination:

Sections examined from the mass revealed invasive cords and collapsed tubules of moderately anaplastic ductal epithelial cells associated with 20% DCIS, solid pattern with lobular cancerization supported by fibro-collagenous disorganized stroma with moderate mononuclear inflammatory cells.

No evidence of perineural invasion or vascular tumour emboli.

The intervening breast tissue showed extensive fibrocystic changes.

The resection margins were free.

The axillary lymph nodes showed reactive changes free of deposits 0/24.

Diagnosis:

Breast mass, wide local excision, axillary lymph nodes;

- Invasive duct carcinoma GII, 20% DCIS, no perineural invasion or vascular emboli.
- Free resection margins, free axillary lymph nodes 0/24.
- Stage pT1 N0 (maximal tumor dimension 1.1 cm).

Prof. Dr. Amal Haridy

د. أمال حوري

Prof. Dr. Essam Ayad

د. عصام عياد

ImmunohistoChemistry report

Report Number: m7186

Female	النوع	امال عبد العظيم محمد	اسم المريض
القاهرة	العنوان	48 Y	السن
03/12/2019 10:20	تاريخ استلام العينة		تاريخ اخذ العينة
25/12/2019 09:17	تاريخ التقرير	مركز اورام السلام	طالب التحليل
		مركز اورام السلام	عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu (ER&RP : Harvey scoring system - Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (E2) Rabbit monoclonal primary antibody.
Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit as ventana benchmark GX.

Result :

ER: NO nuclear staining of : tumour cells

PR: NO nuclear staining of : tumour cells

HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.

Referral Lab :

Comment :

Conclusion :

ER: NEGATIVE

PR: NEGATIVE

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

Pathologist:

د/أينس رمزي

Lab Director: د/ نريزا ماهر



Patient Name: Amaal Abd Al Azeem Mohamed Adawy	Date : 27/01/2024
Exam : C.T CHEST	Age : 52 Year(s)
Medical ID: 1973/2019	

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Reported case of right breast carcinoma underwent right BCS
- Both breasts are partially out of field, right operative bed parenchymal distortion yet no operative bed suspicious masses or collection
- No pathological axillary LNs.
- Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at right upper lung lobe measuring 6x3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

مختبرات أورام
NASSER INSTITUTE
ONCOLOGY CENTER

معهد ناصر
مركز الأورام



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Amal Abdelazim Mohamed.
Referring physician: Prof.

Date: 9/5/2020

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- No definite evidence of metatstatic osseous deposits.

Thanks for reference

Huda Fathy, MD

Nuclear Medicine Unit
Walid Omar, M.D.

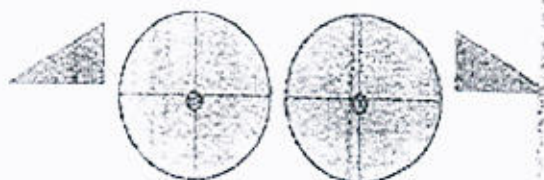


مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

النسب المصرية
طهارة
مصر

MDT FORM

Name:	Dr. Mohamed Elmaghrabi
Age:	50
MRN:	
ID:	
Co-morbidity:	+ T1



Clinical Data

Sex:	SM:
History:	Metastasis Workup:
CR:	
PR:	
HER2:	
KITV:	

T N M

COMMENT:

Grade of Rt breast ca → Rt CRS

→ Adjuvant: 4 AC + 12 w Taxol
Weekly Herceptin & PORT

→ on Tamoxifen → Endometrial
hyperplasia → femoral

MDT OUTCOME & PLAN

--	--	--	--

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
------------------	---------	-----------	-----------

Patient's Name:	Amal Abdelazim Mohamed Adwi
Date:	2026-05-16
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Few scattered pulmonary nodules the largest in the middle lobe measuring 7 mm.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Few scattered pulmonary nodules for follow up.
- ❖ No hepatic metastatic lesions.

Dr.Assem Ahmed

Patient's Name:

AMAL A. EL AZIM MOHAMED

Date:

2026-04-22

Referred by:

Prof. Dr.

Hospital:

Goustave Roussey International Egypt

10/10/2026
10/10/2026
10/10/2026

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ, posterior third architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- **ultrasound** demonstrated UOQ posterior third area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-therapeutic sequelae.

Left breast (BIRADS 1): Normal study.

Recommendation:-

- **For annual follow up.**

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

جمهورية مصر العربية
بطاقة تحقيق الشخصية



امثال
عبد العظيم محمد عدوى
١٢ ش غرب القشلاق - ح البساطي
الوايلي - القاهرة

١٩٧١/١/٢٣ ٢٧١ ١١ ١٣ ١٣ ٠٠٧ ٤١

GH1120146





٢٠١٥/١١

٢٧١١١٣١٣٠٠٧٤١

حاصله على دبلوم صنایع



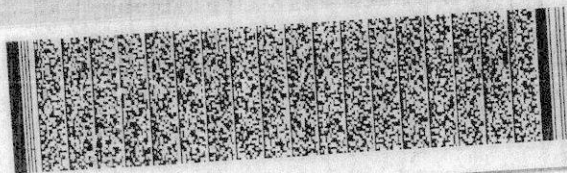
متروجة

مسلمة

أنثى

ناصر رجب عبد الحميد حسن

البطاقة سارية حتى ٢٠٢٢/١١/١٦





Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m7186

Female	: النوع	امال عبد العظيم محمد	: اسم المريض
القاهرة	: العنوان	48 Y	: السن
03/12/2019 10:20	: تاريخ استلام العينة		: تاريخ اخذ العينة
25/12/2019 09:17	: تاريخ التقرير	مركز اورام السلام	: طالب التحليل
		مركز اورام السلام	: عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu (ER&PR : Harvey scoring system - Her2-neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :

ER: NO nuclear staining of tumour cells

PR: NO nuclear staining of tumour cells

HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.

Referral Lab :

Comment :

Conclusion :

ER: NEGATIVE

PR: NEGATIVE

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

Pathologist:

د/أيناس رمزي

Lab Director: د/ تريزا ماهر

Name: amal abd elazeem mohamed

Date: 10 May 2021

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVE, bulky uterus for age measuring about: 9x5x5cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows posterior wall intramural fibroid causing bulge of its wall measures 25x20mm
- The endometrial line is 13mm with cystic changes.

○ **Ovaries: -**

- Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

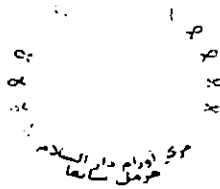
Conclusion:

Endometrial thickening ?endometrial hyperplasia with cystic changes

Uterine fibroid

Best regards

Dr / NEERAJEN RA. HADAN MD



Patient name: امال عبدالعظيم محمد

Date: 01/06/2021

Combined bilateral breast mammography and complementary ultrasound

Indication:

Patient is 49 years old with right CBS for follow up

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)

Right breast :

- global asymmetry with architectural distortion in upper outer quadrant with skin thickening (postoperative changes) , no calcification or mass lesion seen

Left breast :

- No suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound no cystic or solid mass lesions.
- Normal Skin thickening , and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific axillary LN.

Opinion:

- Status post-operative right CBS shows postoperative changes (BIRADS 2)
- Left normal study (BIRADS1)

Yours truly

Dr\ Walaa Abdel _aziz

Breast imaging and intervention consultant

Dar El-Salam



مركز أورام دار السلام

Cancer Center
Radiology Department
قسم الأشعة

Name: *amul Abdelazeem mohamed*

Date : *15/02/21*

Plain X-ray-Chest (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*

Yours truly
Dr. Nermeen Ramadan.MD

Name: amr al abdelazeem mohamed
Date: 15 February 2021

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

Liver: -

- Enlarged in size (17cm at RTMCL) displaying homogenous bright parenchymal echopattern with smooth surface.
- the left lobe shows sub capsular rounded hypoechoic FL measures 15x15mm
- No intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size , homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexal: bulky for age, endometrial thickness = 8mm

Opinion:

Bright hepatomegaly likely fatty

Hepatic FL as described (by previous triphasic CT it is fatty sparing area) for follow up

Bulky uterus

Colonic gaseous distention

Best regards

Dx/nermeen Ramadan, MD

Name : Amal Abdel Azelm

Date : 05/05/2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

I CHEST:

- Right breast edema pattern and heterogeneous parenchyma.
- Few bilateral pulmonary nodules are noted, the largest is seen at the right middle lobe related to the minor fissures measures about 7 mm along its maximal axial diameter.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormalities.

II ABDOMEN AND PELVIS:

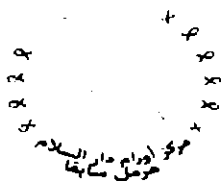
- The liver is of average size showing diffuse low CT attenuation with segment IV sub capsular hypodense focal lesion/area measures about 1.8 cm along its maximal axial diameter.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Bulky uterus with distended uterine cavity.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined part of the skeleton showing no sizable destructive osseous lesions.

Opinion:

- ***Right breast edema pattern.***
- ***Bilateral few small pulmonary nodules, of indeterminate nature for follow-up/further assessment.***
- ***Fatty liver with segment IV hypodense focal lesion/area, for further assessment.***

Dr. Mafimond Saleh, M.Sc

Dr. Amr Farouk, MD



Name : AMAL ABDEL AZIM

Date : 10-06-2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE ABDOMEN & PELVIS WITH TRIPHASIC LIVER ASSESSMENT: -

Known case of cancer breast, US Showed hepatic focal lesion ?? focal fat sparing.

Findings:

- The liver is enlarged in size with diffuse low parenchymal density showing sub capsular patchy and focal areas of alternating hyper & hypodense attenuation likely related to mixed areas of focal fatty sparing & infiltration.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Reviewing skeletal parts revealed lytic osseous lesion with dense sclerotic rim at left femoral trochanteric region.
- Scanned lung bases show left lower lobe 8 mm nodule.

OPINION:

- ***Fatty hepatomegaly sub capsular patchy and focal areas of alternating hyper & hypodense attenuation likely related to mixed areas of focal fatty sparing & infiltration with no definite enhancing focal lesions.***
- ***lytic osseous lesion with dense sclerotic rim at left femoral trochanteric region.***

DR. Ibrahim Elsedawy.M.Sc, FEBR

Dr ,gomaa,md

Name: amal abdelazeem mohamed

Date: 8 June 2020

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

Liver: -

- Average in size displaying homogenous bright parenchymal echopattern with smooth surface.
- Ill-defined geographic hypoechoic area related to the anterior GB wall measures 12x10mm likely fatty sparing area
- No intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- average size, homogenous.

Kidneys: -

- Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.
- No stones or dilated pelvicalyceal system
- No gross lesions

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenxa: no abnormalities could be detected

Opinion:

Bright liver likely fatty

Hepatic FL ?focal fatty sparing area for triphasic CT and follow up

Best regards

Dr/nermeen Ramadan.MD



National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة



Pathology Report

Name: أمال عبد العظيم محمد عدوى Age: 50 years.
Clinical data: Known case of breast cancer
Specimen type: D&C biopsy specimen
Date (Received): 11/8/2021. Date (reported): 19/8/2021
Lab. Number: 1759/ 21 Hospital Number: نفقه

Gross Examination:

Multiple Soft and mucoid tissue pieces admixed with blood, collectively measured 5x5 cm, totally submitted.

Microscopic examination:

Histopathological examination of sections prepared from the D&C specimen received reveals a simple polypoid structure covered by columnar endometrial type of epithelium. The fibro-cellular core contained irregularly dilated endometrial glands, well-formed, thick-walled blood vessels and chronic inflammatory cells. There is no evidence of malignancy in sections examined.

Diagnosis:

Known case of breast cancer; D&C biopsy:

- Hyperplastic endometrial polyp
- No malignancy.

Prof. Dr. Gamal Abdel Atty

Dr. Walaa Ghonaim.



امانة المراكز الطبية المتخصصة
مركز أورام دار السلام
إدارة المعمل
قسم الباثولوجى

Immunohisto Chemistry Report

Name: امال عبد العظيم محمد

Age: 48 years.

Clinical data: Breast mass

Date: 21/6/2020

Primary Sample:

Parrafin block.

Test:

Immunchistochemical studies was done and the results was:

ER: Positive nuclear staining.

PR: Negative nuclear staining.

Conclusion:

ER: Positive (5/8).

PR: Negative.

Prof. Dr. Gamal Abd el-Aty.

د. جمال عبد العالى
Dr: Marwa Mahmoud.

Dar El-Salam



مركز أورام دار السلام

Cancer Center
Radiology Department
قسم الأشعة

Name: amal abelazeem

Date : 07/12/20

Plain X-ray Chest (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

Yours truly
Dr. Nermeen Ramadan.MD

Name; amal abdelazeem

Date: 7 December 2020

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

Liver: -

- enlarged in size (17cm at Rt MCL) displaying bright parenchymal echopattern with smooth surface.
- The left lobe shows well defined hypoechoic FL at segment II measures 15x10mm
- No intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size , homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Empty

Uterus and adnexal: bulky uterus with endometrial thickness=8mm

Opinion:

Bright hepatomegaly likely fatty

Hepatic FL for triphasic CT

Bulky uterus with mild endometrial thickening for follow up

Colonic gaseous distention

Best regards

Dx/nurmeen Ramadan, MD

Dr. IBRAHIM HASSAN

Ph.D. (PATHOLOGY)

دكتور
إبراهيم حسن
دكتوراه في علم الباثولوجيا والأورام
استاذ التحاليل الباثولوجية
بكلية طب الأزهر

Name: أمال عبد العظيم محمد

Date: 15/6/2019

Referred by: Dr. Anas Abdel-Aziz

Ref. No.:

Nature of the specimen: Right upper outer quadrant breast mass, US-guided tru-cut biopsy. Female patient aged 48 years.

Gross description: A single core of tissue measuring 1cm long.

Microscopic examination: Sections reveal infiltration of fibrofatty tissue by irregular groups of malignant epithelial cells exhibiting moderate nuclear anaplasia, mitotic figures and tubular differentiation. The stroma is desmoplastic.

Diagnosis: Right breast mass, tru-cut biopsy: Infiltrating duct carcinoma, grade II.

N.B. Detection of estrogen and progesterone receptor status, HER-2/neu oncoprotein and MIB-1 (Ki-67) is available on paraffin sections on request.

Signature

معمل : 25922312

27865233

معمل : برج النصر - ميدان باب الشعريه

NASSER INSTITUTE
ONCOLOGY CENTER

معهد ناصر
مركز الأورام



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Amal Abdelazim Mohamed.

Date: 9/5/2020

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP;

Report:

- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- No definite evidence of metastatic osseous deposits.

Thanks for reference

Huda Fathy, MD

Nuclear Medicine Unit
Walid Omar, M.D.

DR.
Osama Abdalla Dawoud

Prof. of Diagnostic Radiology
Zagazig University
Fellowship - Karolinska Institute
Stockholm - Sweden



دكتور
أسامة عبدالله داود

أستاذ الأشعة - طب الزقازيق
زميل جامعة كارولنسكا
استوكهولم - السويد

Patient Name : Mrs. Amaal Abdel Azim

Prof. Dr.

Dear Prof. Dr. :

Mammography (cranio-caudal & oblique views)

- = *Rt. outer quadrant ill defined radio-dense lesion.*
- = *No speculated lesion at both breasts.*
- = *No pathological calcifications.*
- = *Increased thickness of right breast skin.*
- = *Retracted right nipple .*

COMPLEMENTARY BREAST U.S.

RT BREAST;

- = *At the RT. upper outer quadrant and axillary tail region , opposite 10 O'clock, there is well defined anechoic elongated cystic lesion with partially turbid fluid content and thick walls.*
- = *It measures 65 x 22 mm.*
- = *It is associated with surrounding edematous changes in form of increased parenchymal echogenicity and intervening minimal fluid.*
- = *Diffuse Increase in skin thickness reaching its maximum at the lower Quadrants measuring 7 mm .*
- = *No ductal dilatation.*
- = *No speculated solid lesions.*
- = *No axillary LN enlargement.*

LT BREAST;

- = *Normal US finding.*

Conclusion;

- ***RT. breast abscess with surrounding edematous changes***
- ***No speculated solid lesions .***

Thanks for your reference

DR. Osama Dawoud



اسم

Name : Amal Abdel Azeim

Date : 05/05/2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

I CHEST:

- Right breast edema pattern and heterogeneous parenchyma.
- Few bilateral pulmonary nodules are noted, the largest is seen at the right middle lobe related to the minor fissures measures about 7 mm along its maximal axial diameter.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormalities.

II ABDOMEN AND PELVIS:

- The liver is of average size showing diffuse low CT attenuation with segment IV sub-capsular hypodense focal lesion/area measures about 1.8 cm along its maximal axial diameter.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Bulky uterus with distended uterine cavity.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined part of the skeleton showing no sizable destructive osseous lesions.

Opinion:

- **Right breast edema pattern.**
- **Bilateral few small pulmonary nodules, of indeterminate nature for follow-up/further assessment.**
- **Fatty liver with segment IV hypodense focal lesion/area, for further assessment.**

Dr. Mahmoud Saleh, M.Sc

Dr. Amr Farouk, MD

Kindly we need to pattern of enhancement of this Hepatic focal lesion.
- showing no enhancement in the portal phase for u/s correlation.



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)

Doctor's Orders أوامر الطبيب

التشخيص: الحمل السابق
اسم المريض: Adnan
رقم الملف: Adnan
نوع التغذية: Adnan

Chest: Adnan

Temp: Adnan

Oral mucositis: Complicated by Adnan Adnan

Canola site infection: Under Adnan

Adnan Adnan

Adnan

Adnan
Adnan
Adnan

بسم الله الرحمن الرحيم
شكر الله
مركز أورام دار السلام
طبيب الأورام

Ad 17/11/2019

توقيع الطبيب:

Chest: Adnan

Temp: Adnan

Oral mucositis: Adnan

Canola site infection: Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

Adnan + Adnan + Adnan + Adnan

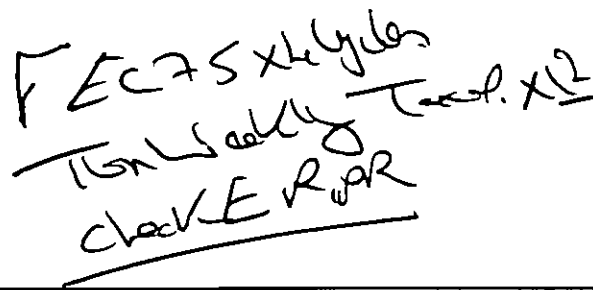
Clinical pharmacy

15/11/2019

التاريخ:

توقيع الطبيب:

الوقت:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هـرمل سابقاً)

التشخيص: الحب الشباب اسم المريض:

نوع التغذية: رقم الملف:

[illegible]

م / ا حال ع / المظفر

جميع الأوامر الطبية لابد ان تكون واضحة وموقعة من الطبيب

512
C/141 A/KV



وزارة الصحة والسكان
امانة المستشفيات الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name: أمال عبد الطيف

MR AM
19

Name:

Date: 1/2/2024

EF	63%	55-75%	PWT		0.6-1.1cm
LVEDD	4.3	3.5-5.8cm	AO.R	3.5	2-3.7cm
LVEDS	2.9	2-4cm	LA	3.4	2-4cm
FS	34%	25-45%	RV		0.7-2.3cm(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

D/ Mohamed Ali

Msc cardiology



Patient Name: Amaal Abd Al Azeem Mohamed Adawy	Date : 27/01/2024
Exam : CT CHEST	Age : 52 Year(s)
Medical ID: 1973/2019	

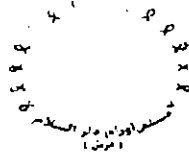
- The scanned skeleton shows left femoral mixed lytic sclerotic lesion
- No vertebral structural collapse or extra-osseous soft tissue component

OPINION:

- ***Reported case of right breast carcinoma underwent right BCS***
- ***Multiple scattered pulmonary nodules likely benign in nature for follow up.***
- ***Left femoral mixed lytic sclerotic lesion, for bone scan correlation.***
- ***No definite hepatic metastatic deposits on non contrast phases.***

Dr. Rasha Elmaghrabi, FRCP, FEBR

Dr. Ola Magdy, M.D.



Patient Name: Amaal Abd Al Azeem Mohamed Adawy	Date : 27/01/2024
Exam : C.T CHEST	Age : 52 Year(s)
Medical ID: 1973/2019	

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

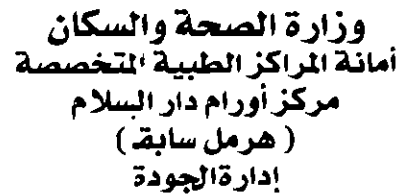
1. Chest:

- Reported case of right breast carcinoma underwent right BCS
- Both breasts are partially out of field, right operative bed parenchymal distortion yet no operative bed suspicious masses or collection
- No pathological axillary LNs.
- Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at right upper lung lobe measuring 6x3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No extra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra renals.
- Smooth filling of the urinary bladder with no filling defects or circumferential outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. Unscanned skeleton:



Handwritten notes:

- Ward
- 11/15
- 11/15



gluery demand
mtataru
MUT pelic km

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

9/2026

اسم المريض :
الرقم الطبي :
التاريخ : 9/2026

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

ABC, blood

Canula - reported

1/8/2026 يومياً لمدة 2

1/8/2026 يومياً لمدة 2

التاريخ:
الوقت:

توقيع الطبيب:
[Signature]

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

Dr. R

framara

one-cphc

zalcum

zante 4 ang

التاريخ:
الوقت:

توقيع الطبيب:
[Signature]

9/11/2026
9/11/2026

9/11/2026
9/11/2026



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي : ١٩٩٩٩٩٩٩٩٩

الأوامر

chest:.....

Temp:..... 37.1

Oral mucositis:.....

Canula site infection:.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

توقيع الطبيب :

التاريخ :

الوقت :

[EF: 63% → 1/2/2024]

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

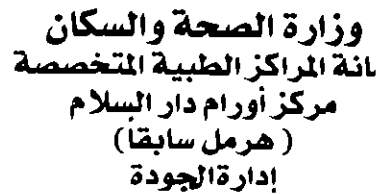
التاريخ :

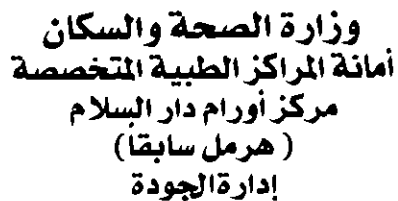
الوقت :

توقيع الطبيب :

.....

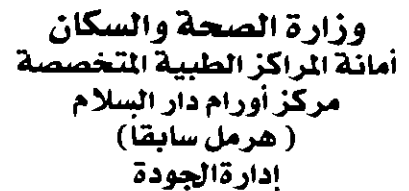
.....



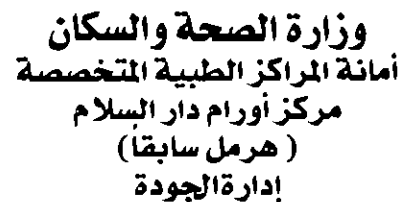


اسم المريض :
الرقم الطبي :
19/11/2019

التاريخ: / /
الوقت:



الوقت: ٤٠



اسم المريض : أمال عبد الرؤوف
الرقم الطبي : ١٩٧٣ / ١٩

التاريخ: ١٦ / ١١ / ٢٠٢٠
الوقت:

التاريخ: ٢٠٢٣ / ١ / ١٢
الوقت:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض : اسم الطبيب :
الرقم الطبي : ٢٩٧٧١٤٠١٩

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

Bil. & sm → free

CRP → free

US Abd. pel → free

RI Hemura 1X1X90

RI Cal 1X1X90

RI Bone alpha 1X1X90

الوقت:

RI Cal 1X1X90

RI Dell 1X1X90

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

RI Capth 1X1X90

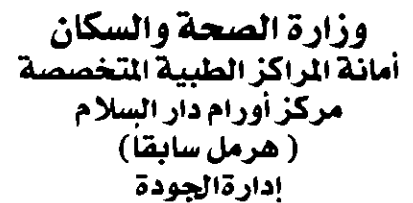
الوقت:

RI Cal 1X1X90

توقيع الطبيب :

التاريخ: / /

الوقت:



chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

فأتم ما أنا عليه
زيت الفلفل الأسود
- فوينا راجع دسوسوف
داريبيج
٥٢
توقيع الطبيب



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:
الرقم الطبي: 1972/10019

الأوامر

chest:.....

Temp:.....

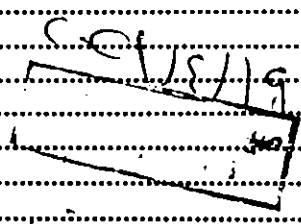
Oral mucositis:.....

Canula site infection:.....

23/3/2021

57-10

14-10-2021



19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

19/4/2021

التاريخ: / /
الوقت:

توقيع الطبيب:
19/4/2021

19/5/2021

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

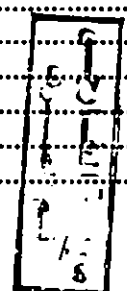
Canula site infection:.....

9/5/2021

For TUS

for echo neck

for echo neck



التاريخ: / /
الوقت:

توقيع الطبيب:
19/5/2021

19/5/2021

Phisamirg
10/5/2021

19/5/2021



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :
19/12/2021

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

11

توقيع الطبيب :
15/12/2021
PHSAmura
التاريخ:
الوقت:

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

12

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

13

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

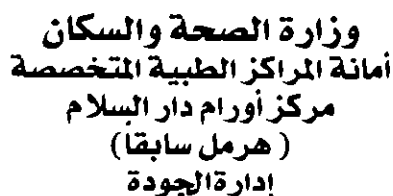
chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....



اسم المريض :
الرقم الطبي : ١٩٧٣/٤٠١٩

Echo next cycle

7. $x_1, x_2, \dots, x_n$ are independent

2. $x_1 \times 1$ 2 $\sim$ 5

$\frac{9}{8} \times \frac{1}{2} = \frac{9}{16}$

o y x c u e l l

توقيع الطبيب

Cliph Doan
11/12/2020

Last Echo on 11/9/2020

4/11/2023

فیرسید دے ک

7. place la réponse

والله اعلم بالصواب

[illegible]

توقيع الطبيب: (باسم) ٢٠١٥

الوقت:

c.c1/1/E
Ph/Samira
4/1/2020



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :
19/12/2020

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

Echo next cycle

7.12.2020

7.12.2020

7.12.2020

7.12.2020

Alphadoc
14/12/2020

توقيع الطبيب

التاريخ:

الوقت:

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

last Echo on 11/9/2020

4/11/2020

C9

7.12.2020

7.12.2020

7.12.2020

7.12.2020

توقيع الطبيب

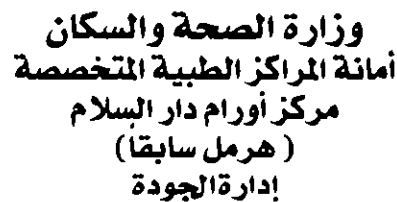
التاريخ:

الوقت:

c.c1/1/8

Ph/Samiya

4/11/2020



11



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:
الرقم الطبي:
أصل الـ (الـ)
1/9/2020

الأوامر

chest:..... 21/9/2020

Temp:.....

Oral mucositis:..... EF 62% on 1/9/2020

Canula site infection:.....

C. Byp

(S) Aely Henept

C5

هيرميند 600 مل كاليه

2 x 1 x 1 C. Byp

هيرميند 600 مل كاليه

د. ب. ب. ب.

C. C. 1/9/21

Philsomira
21/9/2020

التاريخ:
الوقت:

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

C6

EF

هيرميند 600 مل كاليه

2 x 1 x 1 (C. Byp) 2 x 1 x 1

هيرميند 600 مل كاليه

هيرميند 600 مل كاليه

توقيع الطبيب:

التاريخ:
الوقت:

21/9/2020

هيرميند 600 مل كاليه



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:
الرقم الطبي:
أوامر الطبيب:
أوامر

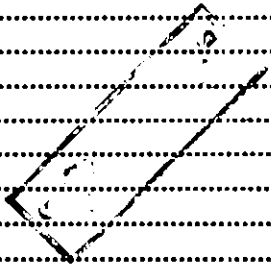
chest: 10/8/2020

Temp:

Oral mucositis: RT Breasted

Canula site infection:

C3 Adj. Hump



كثير من 7- كليم

العنبر 14x2x2

تأثير 2. 1. 1. 2.

الطبيب

11/8/20

التاريخ:
الوقت:

التوقيع:
10/8/2020

الأوامر

chest: 0/8 RR Breasted

Temp:

Oral mucositis:

Canula site infection:

C4 Adj. Hump

كثير من 7- كليم

تأثير 2. 1. 1. 2.

الطبيب

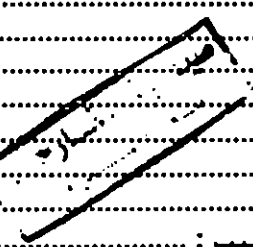
11/8/20

11/8/20

for etho meth

التاريخ:
الوقت:

Clinicalph



التوقيع: الطبيب



وزارة الصحة والسكان
مادة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

أمالي الفيلج

Doctor's Orders أوامر الطبيب

20/7/2020

المريض: Premenopausal Breast cancer
م. الطبي: M. N. H.

Rt BCS الأوامر T₁ N₀ M₀

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

1.1x1 1/2u

ER-ur, PR-ur, Her23

ER+ur 5/8, PR-ur

rec 4AC, 12Tad: 1PORT

C₂ Acy Her

Bilateral SM on 29/6/2020

free R operator bed of ovary

ed, patter wtl Molesden 7

Recurrent

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

شير 7-7, 7-7

تاو كفس 2, 1, 1, 1

V x 3 x C on 1/1/20

1/1/20

1/1/20

التاريخ:

الوقت:

توقيع الطبيب:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

صالح بن النخيل

Doctor's Orders أوامر الطبيب

اسم المريض : perimena
الرقم الطبي : perimena

الأوامر

chest: EF: 57%
Temp:
Oral mucositis:
Canula site infection:

C1 Adj

هرمل 7-10 كس، كس

Vaccination

الفئة Vaccination

داوية

6/29

Repet ER, PR at

/ / التاريخ:
الوقت:

perimena

29/6/2020

ph/samir 29/6/2020

الأوامر

chest: 20/7/2020
Temp:
Oral mucositis:
Canula site infection:

Repet of ER, PR at

ER +ve 5/8

PR -ve

/ / التاريخ:
الوقت:

توقيع الطبيب :



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :
أصل من الأمل

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

U/S HFL

sp an

for Tropic CT

د/بنا

7/18

توقيع الطبيب :

التاريخ: 15-6-2020

الوقت: Pt note Tropic CT. on 10-6-2020

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

الأوامر L to fully hepatomegaly

lytic osseous lesion in left femoral head

Bone scan: free

for Repeat CT, pR at

done 7/18

توقيع الطبيب :
Hemepe: Not available

التاريخ: / /

الوقت:

د/بنا

7/18



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders , أوامر الطبيب

اسم المريض :
الرقم الطبي :
19441009

الأوامر

chest: 4/5/2020

Temp:

Oral mucositis: O/E: NAD

Canula site infection:

prenewp, Bactra - J. No. Mo on 13/7/2020

CR=ur, PR=ur, Her 2-3+

4 AC, 12 weekly Taxol,

PORT 4500 / 20 Fr / 4 Fr-

started on 20/4/2020

توقيع الطبيب :

التاريخ: / /
الوقت:

محمد زار الربيعي

الأوامر

chest: for CT & Bones can

Temp:

Oral mucositis:

Canula site infection:

for CT & Bones can - Herceptin Notable

Bone Scan - free

Labs ok

CT 57% lipodex

CTs -> HFL + fresh L

توقيع الطبيب :

التاريخ: / /
الوقت:

for fresh details about onkair

Repr: No chest
for CTs could

& Radio dig depart